



Surgical Pathology Laboratory

Pathology Report

Report Date:

Report Date:

Submitted by:

Telephone:

Location:

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary		
Case Is (circle):	QUALIFIED	UNQUALIFIED
Reviewer Initials		
Date Reviewed		

CLINICAL INFORMATION:

Bladder Ca.

Specimen submitted: A. Distinguish between ureter or vein - FS; B. Uterus, bilateral tubes and ovaries, bladder

DIAGNOSIS:

A. Soft tissue (excision):

High-grade urothelial carcinoma, present within adventitia of vein.

B. Bladder, uterus, fallopian tubes and ovaries (resection):

High-grade urothelial carcinoma of the bladder with striking discohesive growth pattern, (7.0 cm).

Immunohistochemistry performed at tumor cells:

demonstrates the following staining profile in

Positive - CK7, CK903, mCEA

Negative - CK20, p63, p53, ER, E-Cadherin

Mucicarmine stain highlights presence of mucin within intracytoplasmic vacuoles.

The carcinoma diffusely infiltrates the full thickness of the bladder, extending into the perivesical fat, endomyometrium and cervix, extending to bilateral ovaries and fallopian tubes.

Tumor is present at the anterior and posterior surgical resection margins of bladder, and at the fibromuscular tissue at bilateral ureteral margin sites.

The urethral margin is negative for tumor.

Extensive lymphovascular invasion is present.

TNM Pathologic tumor stage: pT4a pNX

100-3 carcinoma, urothelial, NOS
 8/20/3
 site: path: bladder, NOS (67.9
 Conf: bladder, wall, posterior
 7/23/12 en (67.4

Note:

The tumor is composed of poorly differentiated rather small cells with discohesive growth pattern, intracytoplasmic lumina and increased mitoses. This morphology mimicks that of lobular carcinoma of the breast, and the growth pattern is similar to that seen in poorly-differentiated adenocarcinoma of the stomach, diffuse-type.

The immunoprofile supports the urothelial origin in this case. In addition, conventional high-grade papillary urothelial carcinoma was identified in the TURB specimen. No residual typical urothelial cell carcinoma was identified in sections of the current specimen.

Given the growth pattern of this tumor, its presence in the ovaries and fallopian tubes is interpreted as local spread from primary site, rather than metastatic disease.

Urothelial carcinoma with discohesive growth pattern mimicking lobular carcinoma of breast have been previously reported.

Reference: Transitional cell carcinoma of the bladder mimicking lobular carcinoma of the breast: a discohesive variant of urothelial carcinoma.

Clinical correlation is recommended.

GROSS DESCRIPTION:

A. Received fresh, the specimen is labeled "distinguish ureter or vein," and consists of a segment of vessel measuring 0.5 cm in diameter and 0.2 cm. The specimen is entirely submitted for frozen section diagnosis. Summary of sections:

B. Received fresh, the specimen is labeled "uterus, bilateral tubes and ovaries and bladder," and consists of a cystectomy specimen with attached uterus, bilateral fallopian tubes and ovaries, measuring 10.0 x 9.0 x 8.0 cm overall and weighing 247 grams. The bladder measures 7.0 x 7.0 x 6.0 cm. The anterior and posterior surfaces of the bladder are inked black. The mucosal surface of the bladder is thickened and irregular, and there is diffuse thickening of the bladder wall, ranging in thickness from 1.2 to 3.0 cm. The tumor extends into the perivesicle fat multifocally and approaches the anterior and posterior surgical resection margins. The tumor also extends into the wall of the uterus and cervix which is adherent to the posterior surface of the bladder. The uterus measures 4.0 cm from cornu to cornu, 5.0 cm from cervix to fundus and 2.5 cm from anterior to posterior. The myometrium is fibrotic. The endometrium is unremarkable. The cervix measures 2.5 x 2.0 x 2.0 cm. The left ovary measures 3.0 x 1.7 x 0.8 cm and has a 1.5 x 1.2 x 1.0 cm smooth cyst filled with clear fluid. The left fallopian tube measures 6.0 x 0.8 cm, has a normal fimbriated end and is partially disrupted. The right ovary measures 2.5 x 1.5 x 1.0 cm and the right fallopian tube measures 5.0 x 0.5 cm. The right tube and ovary are unremarkable. Representative sections are submitted. Summary of sections: B1-urethral margin; B2-left ureter margin; B3-possible right ureter margin; B4-left anterior bladder; B5-right anterior bladder; B6-trigone; B7-left posterior bladder; B8-right posterior bladder; B9 to B10-bladder tumor to uterus and cervix; B11-anterior cervix; B12-posterior cervix; B13-anterior endometrium; B14-posterior endometrium; B15-left ovary with cyst and fallopian tube; B16-right ovary and fallopian tube; B17-additional tumor section.

INTRAOPERATIVE DIAGNOSIS:

A. Frozen section diagnosis: Cluster of high grade urothelial carcinoma in the adventitia of vein. (Reported to:

*** Electronically Signed Out ***

Attending Pathologist

The attending pathologist whose signature appears on this report has reviewed the diagnostic slides, and has edited the gross and microscopic portions of the report in rendering the final diagnosis.

Slides: A-3, B-17